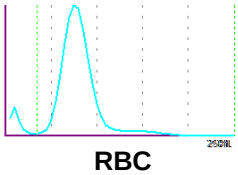
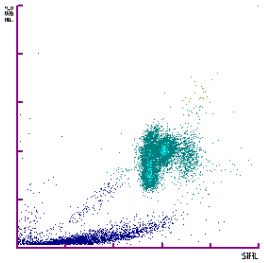
Name	: **Ms. ANJULA SHARMA**	**Age**	: **50 Years**
Lab No.	: **391987852**	**Gender**	: **Female**
Ref By	: **SELF**	**Reported**	: **29/03/2024 09:08:56 PM**
Collected	: **29/03/2024 12:43:00 PM**	**Report Status**	: **Final**
A/c Status	: **P**		
Collected At	: **Indore CC - Rajendra Nagar** **122 - B Rajendra Nagar - Indore 452010 M.P. IND.**	**Processed At**	: **CPL** **Omega Tower Indore**

Test Report

Test Name	Result	Unit		Biological Reference Interval
COMPLETE BLOOD COUNT (CBC) (Electrical Impedence, Photometric)				
Hemoglobin	**9.80**	g/dL	(Spectrophotometry)	12.00 - 15.00
Packed Cell Volume (PCV)	**32.20**	%	(Analogical Integration)	36.00 - 46.00
RBC Count	4.16	mill/mm3	(Electrical Impedence)	3.80 - 4.80
MCV	**77.4**	fL	(Calculated)	83.00 - 101.00
MCH	**23.6**	pg	(Calculated)	27.00 - 32.00
MCHC	**30.4**	g/dL	(Calculated)	31.50 - 34.50
Red Cell Distribution Width (RDW)	**16.5**	%	(Calculated)	11.60 - 14.00
Total Leukocyte Count (TLC)	8620	/cmm	(Electrical Impedence & Flow)	4400 - 11000
Differential Leucocyte Count (DLC)				
Segmented Neutrophils	62.20	%	(Flow Cytometry)	40.00 - 80.00
Lymphocytes	29.50	%	(Flow Cytometry)	20.00 - 40.00
Monocytes	6.60	%	(Flow Cytometry)	2.00 - 10.00
Eosinophils	1.50	%	(Flow Cytometry)	1.00 - 6.00
Basophils	0.20	%	(Flow Cytometry)	<= 2.00
Absolute Leucocyte Count (Calculated)				
Neutrophils	5362	/cmm	(Calculated)	1800 - 7800
Lymphocytes	2543	/cmm	(Calculated)	1000 - 4800
Monocytes	569	/cmm	(Calculated)	120 - 1200
Eosinophils	129	/cmm	(Calculated)	0 - 450
Basophils	17	/cmm	(Calculated)	0 - 100
Platelet Count, Whole Blood	392	thou/mm3	(Electrical Impendence)	150.00 - 450.00
MPV (Mean Platelet Volume)	10.60	fL	(Calculated)	6.50 - 12.00


RBC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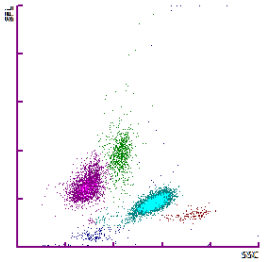
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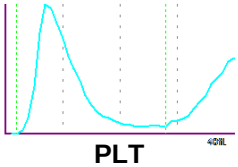
PLT

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Test Report

Note

1. As per the recommendation of international council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood.
2. Test counducted on EDTA whole blood.

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Name	: Ms. ANJULA SHARMA	Age	: 50 Years
Lab No.	: 391987852	Gender	: Female
Ref By	: SELF	Reported	: 29/03/2024 08:45:00 PM
Collected	: 29/03/2024 12:43:00 PM	Report Status	: Final
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Test Report

Test Name	Results	Units	Bio. Ref. Interval
KIDNEY PANEL; KFT, SERUM			
CREATININE, SERUM (Jaffe Compensated)	0.95	mg/dL	0.6 - 1.4
GFR, Estimated (Calculated)	72.99	ml/min/1.73m	> 59
GFR Category	G2		
Urea (Urease UV)	22.10	mg/dL	15 - 40
Urea Nitrogen Blood (Urease Colorimetric)	10.33	mg/dL	6.00 - 20.00
BUN/Creatinine Ratio (Calculated)	10.87		
URIC ACID, SERUM (Enzymatic Colorimetric)	4.30	mg/dL	2.4 - 5.7
Total Protein (BIURET)	7.42	g/dL	6.4 - 8.3
Albumin (BCG)	4.29	g/dL	3.5 - 5.2
Globulin (Calculated)	3.13	g/dL	2.00 - 3.50
A : G Ratio (Calculated)	1.37		
Calcium, Serum (NM-BAPTA)	9.90	mg/dL	8.6 - 10
Phosphorus (Molybdate UV)	**5.21**	mg/dL	2.6 - 4.5
Sodium (ISE)	**134.70**	mEq/L	136 - 145
Potassium (ISE)	4.58	mEq/L	3.5 - 5.1
Chloride (Indirect ISE)	98.10	mEq/L	98 - 108

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Test Report

Note:

- Estimated GFR (eGFR) calculated using the 2021 CKD-EPI creatinine equation and GFR Category reported as per KDIGO guideline 2012.
- eGFR category G1 or G2 does not fulfil the criteria for CKD, in the absence of evidence of kidney damage
- The BUN-to-creatinine ratio is used to differentiate prerenal and postrenal azotemia from renal azotemia. Because of considerable variability, it should be used only as a rough guide. Normally, the BUN/creatinine ratio is about 10:1

- If estimated GFR is 60 to <90:

Advise: CKD Risk Map (Z1014)

- If estimated GFR is <60:

Advise: 1. CKD Risk Map (Z1014) 2. Cystatin C, serum (B173)

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Test Report

Test Name	Results	Units	Bio. Ref. Interval
LIVER PANEL 1; LFT, SERUM			
AST (SGOT) (IFCC without P5P)	25.00	U/L	0 - 32
ALT (SGPT) (IFCC without P5P)	20.30	U/L	<33
GGTP (IFCC)	14.00	U/L	0 - 42
Alkaline Phosphatase (ALP) (IFCC)	89.00	U/L	0 - 98
Bilirubin Total (Diazo)	0.16	mg/dL	0 - 1.1
Bilirubin Direct (Diazo)	0.08	mg/dL	0 - 0.2
Bilirubin Indirect (Calculated)	**0.08**	mg/dL	0.1 - 1.2
Total Protein (BIURET)	7.42	g/dL	6.4 - 8.3
Albumin (BCG)	4.29	g/dL	3.5 - 5.2
Globulin (Calculated)	3.13	g/dL	2.00 - 3.50
A : G Ratio (Calculated)	1.37		

Note :

Enhanced liver fibrosis (ELF) test is used to evaluate liver fibrosis in patients with suspected chronic liver disease due to Viral Hepatitis B & C, Alcoholic liver disease and Non alcoholic fatty liver disease.

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Test Report

Test Name	Results	Units	Bio. Ref. Interval
LIPID PROFILE, BASIC, SERUM			
Cholesterol Total (CHOD-PAP)	**237.40**	mg/dL	Optimal: <200 Borderline high: 200 - 239 High: >=240
Triglycerides (GPO/PAP)	138.30	mg/dL	Optimal : <150 Border Line : 150 - 199 High : 200 - 499 Very High : >500
HDL Cholesterol (HomoGenous Enzymatic Colorimetric)	52.70	mg/dL	40 - 60
LDL Cholesterol,Calculated	**157.04**	mg/dL	<100.00
VLDL Cholesterol (Calculated)	27.66	mg/dL	<30.00
Non-HDL Cholesterol (Calculated)	184.70	mg/dL	<130
CHOL/HDL (Calculated)	4.50		3.3 - 7.1
LDL/HDL (Calculated)	2.98		0.5 - 3.0

Interpretation

NATIONAL LIPID ASSOCIATION RECOMMENDATIONS (NLA-2014)	TOTAL CHOLESTEROL in mg/dL	TRIGLYCERIDE in mg/dL	LDL CHOLESTEROL in mg/dL	NON HDL CHOLESTEROL in mg/dL
Optimal	< 200	< 150	< 100	<130
Above Optimal	-	-	100 - 129	130 - 159
Borderline High	200 - 239	150 - 199	130 - 159	160 - 189
High	>= 240	200 - 499	160 - 189	190 - 219
Very High	-	>= 500	>= 190	>=220

Note:

1. Measurements in the same patient can show physiological & analytical variations.Three Serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL& LDL Cholesterol.

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Test Report

2. As per NLA-2014 guidelines,all adults above the age of 20 years should be screened for lipid status. Selective screening of children above the age of 2 years with a family history of premature cardiovascular disease or those with at least one parent with high total cholesterols recommended.
3. Low HDL levels are associated with increased risk for Atherosclerotic Cardiovascular disease (ASCVD) due to insufficient HDL being available to participate in reverse cholesterol transport, the process by which cholesterol is eliminated from peripheral tissues.
4. NLA-2014identifies Non HDL Cholesterol(an indicator of all atherogeniclipoproteins such as LDL,VLDL,IDL, Lpa, Chylomicron remnants)along with LDL-cholesterol as co- primary target for cholesterol lowering therapy. Note that major risk factors can modify treatment goals for LDL &Non HDL.
5. Apolipoprotein B is an optional, secondary lipid target for treatment once LDL & Non HDL goals have been achieved.
6. Additional testing for Apolipoprotein B, hsCRP,Lp(a) & LP-PLA2 should be considered among patients with moderate risk for ASCVD for risk refinement

Treatment Goals as per NLA 2014

RISK CATEGORY	NON HDL CHLOESTEROL (NON HDL-C) (mg/dL)	LDL CHOLESTEROL (LDL-C)(mg/dL)	APOLIPOPROTEIN B (mg/dL)
Low/Moderate/High	<130	<100	<90
Very High	<100	<70	<80

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Test Report

Test Name	Results	Units	Bio. Ref. Interval
IRON STUDIES, SERUM			
Iron (Ferrozine)	55.90	µg/dL	37 - 145
Total Iron Binding Capacity (TIBC) (Spectrophotometry)	**438.70**	µg/dL	200 - 400
Transferrin Saturation (Calculated)	12.74	%	12 - 45

DR. POONAM ARORA MD
PATHOLOGIST

---------- End Of Report ----------

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